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HEALTHY DISCUSSIONS · MAKING THE CASE

Game-based HCP education earns its place in the exam room.

Independent research with 11 HCPs across family medicine, internal medicine, pediatrics, and urgent care.

PREPARED FOR
MEDICAL AFFAIRS · COMMERCIAL · L&D

BASIS
MODERATED HCP RESEARCH, 2025

STATUS
CONFIDENTIAL DISCUSSION DRAFT

THE CASE IN ONE PAGE

When game-based learning is built on real clinical data, designed for HCPs, and deployed to the right audience — **it produces outcomes traditional formats have not delivered.**

+2_{pts}

Confidence gain on a 7-point scale after a single gameplay session

9_{/11}

HCPs rate hesitation to engage with game-based learning at 3/7 or below

9+_{/10}

Likelihood to recommend among HCPs who found the game relevant

10_{/11}

Want game-based learning extended to additional treatments

01

The HCP challenge.

HCPs are not struggling for conviction. They are struggling because the conversations are complex, time-constrained, and emotionally loaded — and their tools have not kept pace.

A DAILY REALITY, IN THREE VOICES

The conversations are already
happening.
The tools haven't caught up.

01

**It comes up every day —
repeatedly.**

"Today, ten minutes ago." Most HCPs no longer raise the topic proactively because patients have already made up their minds.

02

**Resistance isn't monolithic —
but training treats it that way.**

Adverse-event fear, public-figure distrust, social media misinformation, government suspicion. One scripted response can't cover it.

03

**The available tools are falling
short.**

CDC sheets described as "boring and wordy." Journal articles useful for self-study but they don't translate into patient conversation.

SECTION TWO

02

By the numbers.

Independent moderated research with eleven Healthcare Professionals across family medicine, internal medicine, pediatrics, and urgent care.

METRIC 01 · CONFIDENCE

+2 points on a 7-point scale

HCPs with genuine room to develop reported gains of +1 to +2 points in conversational confidence after a single gameplay session.

BEFORE · AFTER

4



6



Largest reported gain. Both +2 movers attributed the shift directly to the game's hesitancy-identification framework and clinical data content.

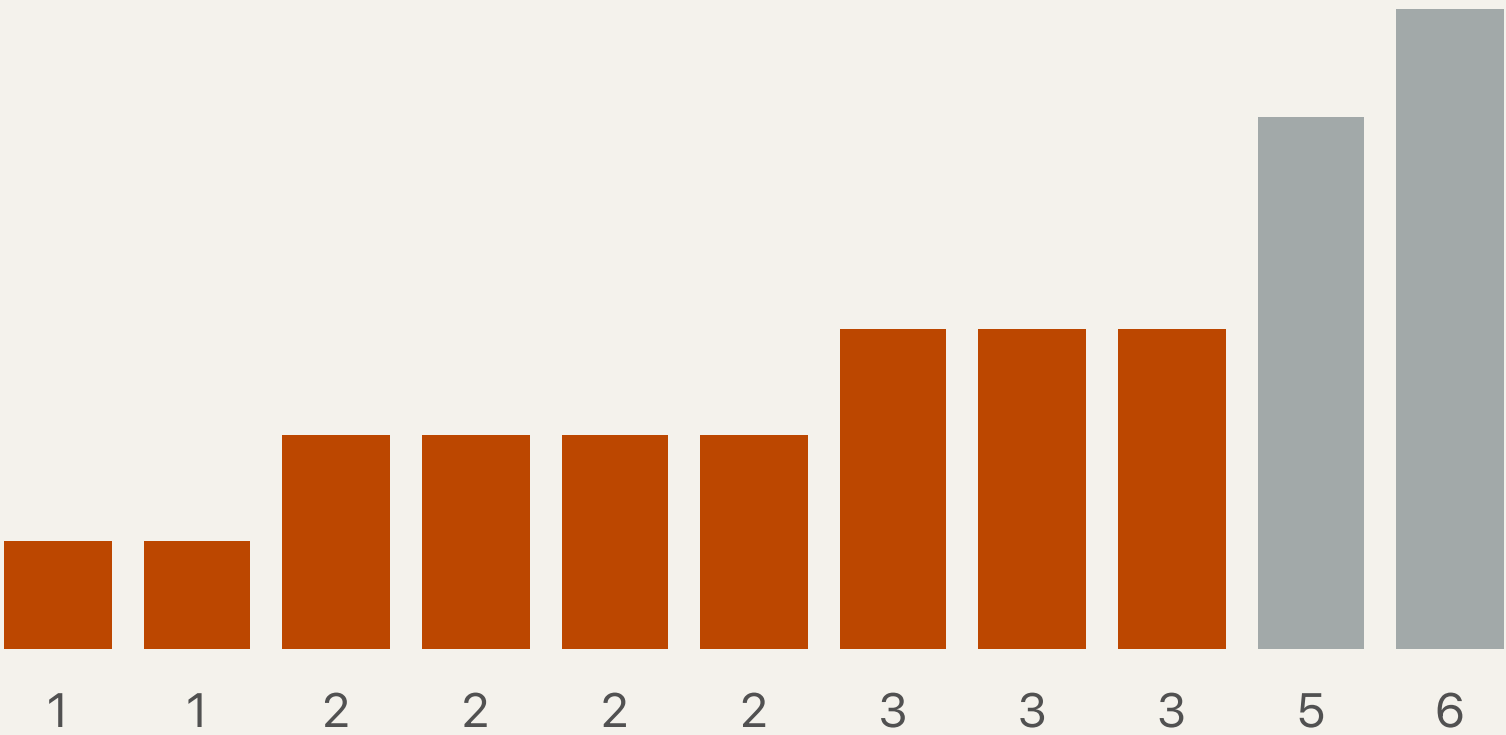
METRIC 02 · RECEPTIVITY

9 of 11 HCPs

Rated their hesitation to engage with game-based medical learning at **3 out of 7 or below**.

The dominant attitude in this group was openness, not resistance — format adoption is not a barrier, it is an opportunity waiting to be activated.

HESITATION · 1 = NONE, 7 = TOTAL



Eleven HCPs, plotted by self-reported hesitation rating. The nine HCPs at or below 3/7 are shown in burnt orange.

METRIC 03 · ENJOYMENT OF DATA

8-10_{/10}

Enjoyment scores across **every participant who completed** the clinical data module — COVID mortality, hospitalization, adverse event rates.

High enjoyment of the data module is a signal of learning engagement, not entertainment. Participants cited the precision of the data and its immediate applicability in patient conversation.

WHY IT SCORED HIGH

Precision of the data

Specific numbers HCPs could cite — not general reassurance.

Immediate applicability

Talking points HCPs intended to use that week, in that practice.

Surprise at the numbers

Genuine new learning — adverse event rates were lower than assumed.

METRIC 04 · LEARNING EFFECTIVENESS

6-7 / 7

Self-rated learning effectiveness from HCPs who completed the game experience.

Participants cited the combination of hesitancy-type identification training and data-driven talking points as the source of this effectiveness. Multiple described the experience as "not just fun — actually informative."

EFFECTIVENESS RATING



1 · Not effective

7 · Highly effective

"Not just fun — **actually informative.**"
Internal medicine physician, recently in practice

METRIC 05 · LIKELIHOOD TO RECOMMEND

9+ /10 average

Among HCPs who found the game relevant to their practice — recommendation scores ranged 7 to 10.

High peer-advocacy intent was consistently linked to the game's clinical realism and the practical value of having precise, retrievable statistics for patient conversations.

WHERE HCPS WOULD SHARE IT

- Early-career colleagues
Navigating their first years of hesitancy conversation.
- Residency program orientations
A repeatable training surface for incoming cohorts.
- Multidisciplinary care teams
Wherever multiple staff members engage with hesitant patients.

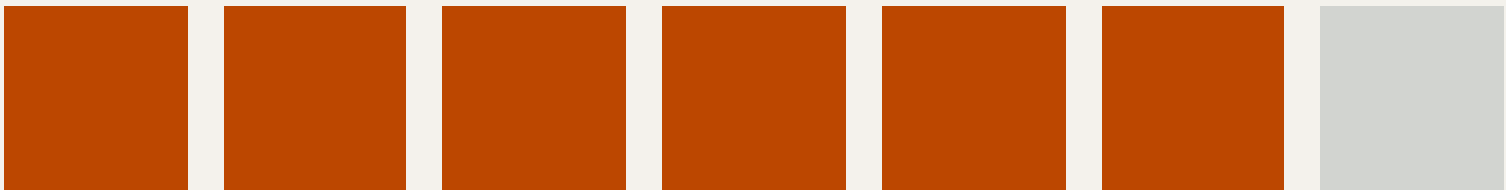
METRIC 06 · FUTURE ENGAGEMENT INTENT

6 of 7 HCPs

Among HCPs who engaged positively with the game, six of seven rated their likelihood to engage with a future game-based learning tool at **6 or 7 out of 7**.

And that intent extended beyond COVID — participants asked for game-based formats across treatments, disease states, and pharmaceuticals in their specialty.

FUTURE INTENT — 6 OR 7 / 7

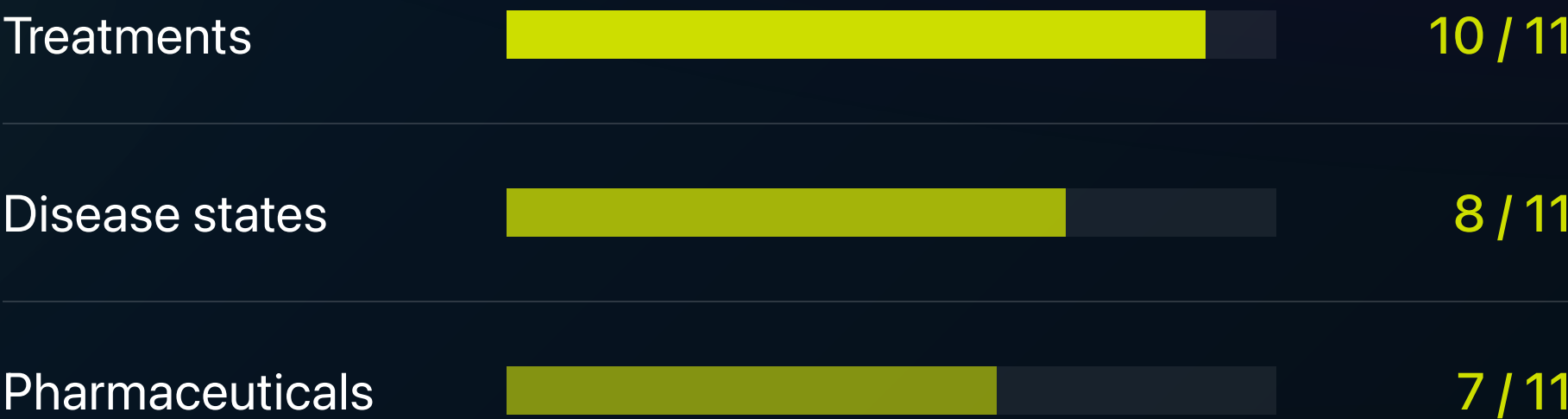


Each square represents one HCP from the engaged cohort.

METRIC 07 · CROSS-TA DEMAND

Near-universal
interest **in the format**
— not just this game.

When asked which other clinical topics
they would want in a game-based format,
HCPs answered consistently and broadly.



HCPs who selected each topic as a desired game-based learning area. Multiple selections permitted.

SECTION THREE

03

What the research shows.

Five findings from eleven moderated sessions — the why behind the numbers.

01

The game builds the conversational skill HCPs need most: **identifying why**.

Before choosing a response, HCPs need to know **which** hesitancy they're dealing with.

The concern-identification framework — mapping patient statements to categories like adverse-event fear, public-figure distrust, and social media misinformation — was described as immediately transferable to real patient conversations.

HCPs with lower baseline confidence reported the clearest gains. Those gains were explicitly attributed to the framework, with the statistical data backing it up.

A family medicine physician described going from a confidence score of 4 to 6 specifically for challenging pediatric vaccine conversations — attributing the gain to what she learned in the game's data and dialogue modules.

02

Clinical data HCPs will actually use **at the point of care.**

COVID hospitalization, mortality, and adverse-event rates by age group — the most consistently valued element across participant types.

HCPs knew these conversations required evidence, but few had precise, retrievable numbers. The game closes that gap.

7/7

Learning effectiveness ratings driven by the statistical layer.

10/10

Enjoyment of data modules — among physicians who described journal articles as putting them to sleep.



One HCP planned to keep the key statistics on a card in her coat pocket.

03

HCPs are receptive —
across experience levels
and specialties.

Physicians, PAs, family medicine, internal medicine, urgent care, early- and senior-career — the same answer.

Clinicians who grew up on video games described the format as a natural fit. Clinicians who did not described it as a welcome relief from journal articles and compliance modules.

A physician assistant with 10 years in urgent care rated her hesitation at 1 out of 7 and described her experience as:

"This is like a real conversation. This is exactly what happens."

04

High recommendation intent becomes a distribution channel.

Among relevant-fit HCPs, recommendation scores averaged above 9 out of 10 — not passive endorsements.

Participants named specific peer contexts where they would share the game: with early-career colleagues, in residency orientations, across multidisciplinary care teams.

An HCP who recommends a learning tool to colleagues is a trusted distribution channel — far more credible than a pharma email or a website banner.

05

Game-based learning addresses a format gap traditional CME has left open.

Existing tools are informative but not engaging — or engaging but not informative. The game occupies the position neither delivers.

TRADITIONAL FORMATS

- Journal articles
- CDC fact sheets
- Webinars (charted-through)
- Static PDF modules

HEALTHY DISCUSSIONS

- Data-rich
- Interactive
- Clinically contextual
- Designed for HCPs

SECTION FOUR

04

In their own words.

Six voices from the eleven HCPs who participated in moderated research sessions.



*Not every hesitant patient is hesitant for the same reason. The game helped me think through how to identify **why** a patient is resistant before jumping to the scripted response. And having actual statistics I could cite in the conversation — that's the difference between a talking point and a convincing argument.*

Internal medicine physician, recently in practice

Reported a +2 confidence gain after completing the game



I actually wanted to write the numbers down. Not to use the game in the exam room — but I'd keep a card in my coat pocket. When a patient says they've heard the vaccine is dangerous, I can pull it out and say: here is the actual rate of adverse events in patients your age.

Specific numbers land differently than general reassurance.

Senior internal medicine physician, private practice

Rated game effectiveness 7 out of 7



*The scenarios aren't hypothetical. These are conversations I have every single week. The hesitancy types the game surfaces, the way the patient pushes back, the social media concern, the adverse event fear — that's real. **It felt like a simulation of my actual practice, not a training module someone designed in a conference room.***

Physician assistant, urgent care + primary care

10 years in practice · Likelihood to recommend 9–10 / 10



*What this tool does well is remind you that patients aren't just 'anti-vaccine.' They have specific fears — misinformation, side effects, distrust of institutions — and the game helps you identify which one you're actually dealing with. **I see this working for everyone on the care team who has these conversations, not just the physician.***

Physician, 35+ years in practice

Effectiveness 7/7 · Likelihood to recommend 10/10



*The data modules changed the quality of the experience. I went from curious to genuinely engaged. I went in expecting the usual talking points. What I got was actual statistics I could remember and repeat — hospitalization rates, adverse event comparisons, vaccine uptake data. **That's what a hesitant patient needs to hear, and now I have it.***

Family medicine physician, mid-career
Likelihood to recommend 7–8 / 10



*I'd share this with colleagues. It reinforces the case for vaccination in a way that is evidence-backed, and the format makes the evidence stick better than reading another journal article. **Learning something through an interactive experience is just different than reading it — I remembered more.***

Family medicine physician, recently in practice
Likelihood to recommend 9 / 10

SECTION FIVE

05

The opportunity.

From a single product to a scalable HCP education channel — what the research enables you to build.

FROM PRODUCT · TO CHANNEL

A format with proven HCP demand —
and portfolio-wide potential.

10 / 11

Treatments

A near-universal request for game-based learning on therapeutic agents.

8 / 11

Disease states

Pathophysiology, diagnosis, staging — the building blocks of clinical judgment.

7 / 11

Pharmaceuticals

Mechanism of action, dosing, evidence — the standing brief of medical affairs.

This is interest in a format, not interest in a single game. For a pharmaceutical company with a portfolio and a standing need to educate HCPs, that signal represents an education channel — not a one-time campaign.

REACHING HCPS WHERE THEY ARE

Mobile-first, self-paced, segment-completable
— designed for the fifteen-minute window
webinars can't fill.

WINDOW 01

Between patients

The unpredictable 10–15 minute gap in a clinic day.

WINDOW 02

Lunch breaks

Mobile-first design fits one-handed phone use.

WINDOW 03

Admin half-days

The Wednesday block that's already protected for learning.

WINDOW 04

Evenings, post-clinical

Self-paced, no scheduled commitment, resumable.

ENGAGEMENT IS THE NEW COMPLETION

Enjoyment isn't a vanity metric — it's a proxy for retention.

TRADITIONAL CME

A completion problem.

- Webinars attended while charting.
- PDF modules opened and closed.
- Journal articles read at varying attention.

A disengaged HCP isn't retaining anything.

HEALTHY DISCUSSIONS

An attention proxy.

- Enjoyment scores of **8–10 / 10** on the educational core.
- Engaged HCPs retain content.
- Learning and enjoyment move in the same direction.

When content is clinically grounded and the format respects HCPs' time, attention follows.

ROI ANCHORED IN BEHAVIOR CHANGE

Confidence in a product conversation translates directly to recommendation behavior.



The confidence gains documented here are not abstract survey scores. They are self-reported shifts in a clinician's readiness to have a specific, high-stakes conversation.

BEHAVIOR 01

Citing specific statistics in patient conversations.

BEHAVIOR 02

Identifying hesitancy type before responding.

BEHAVIOR 03

Framing adverse-event risk in precise, not general, terms.

CONCLUSION · CALL TO ACTION

Let's build the education
channel **your current
investments are leaving on the
table.**

A purpose-built, game-based learning program — for your products and your target HCP audience — delivering outcomes traditional formats have not.

NEXT STEP

**HALF-DAY SESSION WITH YOUR
TEAM**

WE BRING

**THE FULL RESEARCH
RECORD**

YOU LEAVE WITH

**A SCOPED PROGRAM FOR YOUR
PORTFOLIO**